

## **REPORT**

### **REPORT OF A VISIT BY PROFESSOR INAYAT KHAN TO REVIEW THE ACTIVITIES WITHIN THE DIVISION OF DRUG CONTROL AND TRADITIONAL MEDICINE (DC&TMD) IN THE NATIONAL INSTITUTE OF HEALTH, ISLAMABAD. (DECEMBER 3-12, 1996).**

The Executive Director of the NIH had asked me to review the functioning of the various units and sections in DC&TMD and suggest modifications for improvement and in particular to offer closer collaboration with the Ministry of Health in drug control.

#### **METHODS**

All the head of units and sections within DC & TMD were interviewed regarding their responsibilities, work load and future plans. Meetings were also held with heads of other Divisions regarding their collaboration with DC & TMD. I also consulted them on some of the plans I am going to propose of the future. I lectured to students of psychology at Islamabad College for Girls as well at the PAF Finishing School at Chaklala on prevention of substance abuse. These two groups clearly desire information on drug in general.

Drug therapy is an essential element in the health of a nation. For the rational use of drugs, quality, efficacy, safety and price of drugs and the availability of essential drugs are important criteria. The manufacturers of drugs and those who trade in drugs, the profession of pharmacists and the health care professionals as well as the consumers have great responsibility to meet the need of the nation. WHO has great contribution towards optimizing the role of each of the players listed above. The Ministry of Health and other health providers have direct responsibility in this.

In this complex subject process an institution such as the NIH and in particular the Division of DCTM has a great role to play. In the planning of establishing this Division it was envisaged that this laboratory will have jurisdiction on pharmaceuticals as well as traditional medicines since the Government has recognized the systems. This laboratory was to act as an appellate laboratory with regard to the quality of drugs but in addition will offer drug testing facilities in the Northern part of Pakistan. I was personally clear in my mind at that time that this Institute will have mechanisms for post-marketing surveillance of drugs after they were registered. Over the years the facilities of the DC&TMD have improved especially with the contribution of the

Japanese Government. The role of the previous Directors of the laboratory and in particular that of Dr. Fazli is worth appreciation.

### OBSERVATIONS

I will not give a detailed account of my observations but will comment on two aspects.

1. Activities which are worth appreciation.
2. Activities which can be replaced/modified and the staff could undertake new responsibilities. Out of the staff 2 have obtained their Ph.D. degrees and 4 others are pursuing their studies for Ph.D. degrees. This is appreciated. The existence of drug/poison information center is a significant development but its services must now be utilized all over the country.

The facilities for quality control are very good including those for psychoactive substances. Drug reporting in serum for 6 drugs with a promise for another two must be further strengthened and utilized by the medical profession. I was also told that the paper on determination of serum rational in the Journal of Chromatography in 1995 was a good piece of research work. Col. Najmis research for Ph.D. on drug resistant malaria is also a good work. The training of a scientist and the plans for investigating bioavailability of five antibiotics as approved by the Ministry of Health must be taken in hand immediately. The experience of the last eighteen months of drug utilization studies in three university hospitals in Rawalpindi and Islamabad was very much appreciated at the sixth international conference on clinical pharmacology and therapeutics in Buenos Aires in August 1996. Promoting the rational use of drugs is also a significant development in Pakistan and is a unique example of cooperation between the Ministry of Health and the NIH with the assistance of the WHO. Collaboration with the WHO in promoting rationality in the use of traditional medicines is another fine example. On the other hand certain activities for example collection of data on 400 plants seems out of relevance to their immediate therapeutic values. Concentrating only on the level of steroids in traditional medicines without a well planned study of such products without looking into other possible essential constituents for e.g antibiotics, hormones and psychoactive substances is yet another example for improved performance in future.

Studies of constituents in plant material without a well planned objective is also a flaw in the programme. Lack of coordination within the different units and sections and other divisions of the NIH is unfortunate and must be improved because DC&TMD has excellent facilities for chemical and biological studies as well with the possibility of clinical evaluation of drugs which must be exploited.

Pakistan is riddled with drug addiction and the NIH should offer services in chemical analysis of these substances and in particular assisting law enforcing efforts must be further strengthened.

I am also convinced that the NIH in collaboration with Rawalpindi General Hospital can run a course in Urdu for informing religious leaders of the menace of drug addiction by preaching preventive measures.

Collaboration between the NIH and Drug Control Administration in the Ministry of Health is of vital importance. During the planning of projects and day to day activities a mechanism must be established for this type of work.

The following specific steps are proposed:

1. I strongly recommend that the full time Chief of the DC&TMD be in BPS 20. I consider this of vital importance. The morals of the staff and those at the top of their grades needs sympathetic consideration.
2. The activities I propose requires no more water tight walls between the different units and sections. It is also logical that a pharmacognosy expert can assist a team investigating harmful effects of drugs. A scientist (chemist) can lead a team investigating the use and abuse of drugs in the population.
3. A serious effort is needed to develop a project to investigate the quality of pharmaceutical preparations available in the market and its results must be published in scientific journals and presented in meetings of doctors all over the country.
4. Similarly a well planned study of the preparations of traditional medicines available in the market for the presence of steroids, antibiotics, hormones and psychoactive substances be initiated.
5. Investigating and reporting adverse drug reactions initially in the area of Islamabad is a golden idea. This can be done by studies similar to the drug utilization studies conducted in the three hospitals in Rawalpindi/Islamabad. Collaboration with the PCPS in this area will be very useful.
6. The drug utilization studies which involved the three hospitals can be further extended in the these hospitals to other specialties. I am also aware of the wishes of Quetta and Peshawar groups to carry out this work.
7. Acute intoxication studies and investigating the role of drugs in forensic death

cases requires collaboration between the DC&TMD, public health division and forensic experts in Rawalpindi/Islamabad area.

8. Traffic accidents take a heavy toll of life in Pakistan. The NIH can investigate a epidemiological study and by finding drugs in body fluids of injured or dead drivers. This will be a great service for preventive programmes. This requires collaboration with the police and anti-narcotics force.
9. Training staff in UNDCP laboratory in Vienna in the field of psychoactive substances and in particular finger print investigation of narcotic drugs will give the NIH a new area of collaborate with UNDCP and Interpol.
10. Training programme for analysts, drug control officials, doctors and pharmacists in their various fields of expertise must be initiated with the Provincial and educational institutions.
11. The Quaid-e-Azam University is a great institution and active collaboration in the mutual areas of interest must commence in a planned way.
12. Members of the medical profession and the consumers of various services offered by the NIH is another area to be strengthened. It is high time to have discussion with them and collaborate and assist them in the quality control of drugs, bioavailability studies, adverse drug reactions and drug utilization studies.
13. The following are some more specific activities which though going on could be further strengthened and I would advice that one of the technical officers may be made incharge of these activities inspite of his/her technical responsibilities.
  - a. Information collection and dissemination.
  - b. Training i.e. in service training for the staff within the Division as well training for technical people outside of the Division e.g. drug inspectors etc. etc.
  - c. Course on the subject of drugs for the public, students, law enforcement personpel and even politicians who would like to participate. This will be a week long activity at the NIH covering 10-12 lectures in the afternoon or in the evening for which fees could be charged.
  - d. Collaboration with the universities in research and especially with QAU.
  - e. A desk could be created where information on pricing of drugs, be collected though it is the responsibility of the Ministry of Health.
  - f. Smoking cessation course. This may also be considered.
14. The earlier practice of obtaining quality control data of a drug prior to registration by the Ministry of Health must be resumed as a pre-requisite without further delay. This information is absolutely essential before a drug arrives in the market. Registration data must be substantiated within the country by the NIH.